Situation Monitoring Forms

Printable version for manual completion

Simple form

Describe one event. Mark information sources: O — direct observation, P — the person's communication (including AAC), R — another person's report, H — hypothesis. Select the relevant source checkboxes beside each stage description; you may select several. Further details are optional. Missing data does not mean no difficulty. Record when it is safe to do so. "Unknown", "not observed" and "not applicable" mean different things; a blank never means "no".

Basic information

Date ..... **Time** ..... **Place** .....

Person's identifier ..... **Person completing the record** .....

Other people present

.....

Observation start and end

.....

Environment

☐ Home ☐ School

☐ Care setting ☐ Other: .....

Quick situation description

What was the person's state before the situation?

Sleep, hunger, pain, noise, changes of plan, earlier tension.

What happened immediately before the situation?

Describe the preceding minutes: a demand, change, refusal, waiting or contact.

First signs of rising tension

What appeared before the situation intensified: changes in body, voice, movement or contact.

Sources for this description — Early signs

☐ O — direct observation ☐ P — person's communication (including AAC) ☐ R — another person's report ☐ H — hypothesis

Source details (optional) — Early signs

.....

How did the environment respond to the first signs of difficulty? What did the person do or communicate after that response?

Include observations and information communicated by the person. If it is difficult to assess — indicate this.

Sources for this description — Escalation
☐ O — direct observation ☐ P — person's communication (including AAC) ☐ R — another person's report ☐ H — hypothesis

Source details (optional) — Escalation

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How did the escalation unfold? What actions or changes accompanied its intensification and subsiding?
Include observations and information communicated by the person. If it is difficult to assess — indicate this.

Sources for this description — After escalation
☐ O — direct observation ☐ P — person's communication (including AAC) ☐ R — another person's report ☐ H — hypothesis

Source details (optional) — After escalation

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What did the person need before resuming contact or activity? What was provided, and how was readiness recognized?
Include observations and information communicated by the person. If it is difficult to assess — indicate this.

Did the person have influence, choice, room to negotiate or take a break?
What could the person control, and what was imposed or unavoidable?

Was the situation predictable? What was known / unknown?
Was it clear what would happen, how long it would take and when it would end?

Time until signs of readiness were observed
How many minutes after the described behavior ended were signs of readiness observed? You may give clock times. Mark estimate / missing data / not observed by observation end. Describe needs and signs of readiness at the After escalation stage.

Extended form

One record concerns one event. O — observation, P — the person's communication (including AAC), R — another person's report, H — hypothesis. Select the source checkboxes beside each stage description; you may select several. Further details are optional. State when your observation began. Do not guess: enter "missing data" where assessment is not possible. Distinguish unknown, not observed and not applicable.

Basic information

Date ..... **Time** ..... **Place** .....

Person's identifier ..... **Person completing the record** .....

Other people present

.....

Observation start and end

.....

Environment

☐ Home ☐ School
☐ Care setting ☐ Other: .....

0. Baseline and daily context

Tension level before the event

Observer judgement, not a diagnostic scale. Describe specific signs compared with this person's usual presentation; leave the level unrated if unknown. Do not calculate a total.

☐ 0 low / stable ☐ 1 elevated
☐ 2 high ☐ 3 very high

Individual signs supporting the tension rating

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Was the person tired?

☐ Yes ☐ No ☐ Unknown

Did the person get sufficient sleep?

☐ Yes ☐ No ☐ Unknown

Sleep information and its source — if available

E.g. short sleep, waking, nightmares or very early waking.

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Stay stage / daily context, if applicable

☐ arrival / change day ☐ departure / return day
☐ middle of the week ☐ end of the week
☐ not applicable

Was this stage stressful? — residential care, if applicable

☐ Yes ☐ No
☐ Unknown ☐ Not applicable

Contributing stressors

☐ poor sleep / night waking ☐ change of plan
☐ leaving / returning / travel ☐ visitors / more people
☐ conflict ☐ time pressure
☐ noise / crowding ☐ pain / discomfort
☐ hunger / thirst ☐ other: .....

Body state

☐ hunger ☐ thirst
☐ need for toilet ☐ pain / discomfort
☐ physical tiredness ☐ body temperature — cold/hot
☐ muscle tension ☐ other: .....

Sensory conditions

☐ noise / sounds ☐ light
☐ crowding / number of people ☐ smells
☐ textures / touch ☐ room temperature
☐ other: .....

1. Immediately before the event

What happened immediately before the event?

- ☐ activity change
- ☐ waiting
- ☐ request / instruction
- ☐ social contact
- ☐ conflict
- ☐ other: .....
- ☐ end of activity
- ☐ refusal / no access
- ☐ behavior correction
- ☐ noise / crowding
- ☐ transition between places

Details of what happened immediately before

Describe observable facts, without judging or blaming. A preceding event is not an established cause.

2. What was expected at that moment?

What did the person have influence over?

Choice of order, time, place, way of doing the activity or opportunity for a break.

What was unknown, imposed or outside the person's influence?

Sudden change, lack of information, time pressure or no opportunity to refuse or negotiate.

Was it clear what would happen?

☐ Yes ☐ No ☐ Unknown

Was it clear how long it would take?

☐ Yes ☐ No ☐ Unknown

Was there room for negotiation or choice?

☐ Yes ☐ No ☐ Unknown

Expectations / demands

- ☐ no demands
- ☐ starting a task
- ☐ waiting
- ☐ following a rule
- ☐ other: .....
- ☐ end of activity
- ☐ moving to another place
- ☐ social contact
- ☐ self-care task

3. First signs of rising tension

Did signs appear?

☐ Yes ☐ No ☐ Unknown

How long before escalation?

Minutes or clock times; mark an estimate or missing data.

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Observed changes in body and movement

- ☐ faster breathing
- ☐ sweating
- ☐ dilated pupils
- ☐ other: .....
- ☐ muscle tension
- ☐ trembling
- ☐ fleeing / fighting actions

Observed reduction in contact or activity

- ☐ staring at one point / floor
- ☐ slowed movements
- ☐ withdrawal
- ☐ no response to voice
- ☐ body slumping
- ☐ other: .....

Observed responses to surroundings

- ☐ covering ears
- ☐ stereotyped movements
- ☐ leaving the room
- ☐ covering eyes
- ☐ avoiding touch
- ☐ other: .....

Sources for this description — Early signs
☐ O — direct observation ☐ P — person's communication (including AAC) ☐ R — another person's report ☐ H — hypothesis

Source details (optional) — Early signs

.....

How did the environment respond to the first signs of difficulty? What did the person do or communicate after that response?
Include observations and information communicated by the person. If it is difficult to assess — indicate this.

What usually appears first?
The first small sign that may help identify an opportunity for earlier support.

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Did these signs precede the described behavior in other records?
☐ Yes ☐ No ☐ Unknown

3B. Continuing activity, compensation and possible masking

What was observed while the activity continued?
Record actions, available support and time. Continuing alone does not demonstrate overload or masking. Not applicable / missing data are valid answers.

What did the person communicate about effort or discomfort?
The person's communication (P), including AAC. No communication does not mean no experience.

What was noticed later?
Separate direct observation (O) from others' reports (R); give time and source.

Are compensation or masking being considered — on what basis?
A hypothesis (H), not a diagnosis. Insufficient basis is a valid answer. The form itself does not measure hidden costs.

4. Support provided and conditions of access

Actions taken
☐ reducing sensory input
☐ providing predictability
☐ time without demands / pause
☐ other: .....
☐ increasing autonomy / choice
☐ interoceptive support
☐ sensory regulation

Was access to support conditional on following an instruction?
Separate conditions for receiving help, pressure and opportunities for negotiation.
☐ Yes ☐ No ☐ Partly ☐ Unknown

Did the person use this form of support?
☐ Yes
☐ Partly
☐ No
☐ Unknown

5. Behavior description

Sources for this description — Escalation

☐ O — direct observation ☐ P — person's communication (including AAC) ☐ R — another person's report ☐ H — hypothesis

Source details (optional) — Escalation

.....

How did the escalation unfold? What actions or changes accompanied its intensification and subsiding?

Include observations and information communicated by the person. If it is difficult to assess — indicate this.

Magnitude of the described behavior

Give a count, duration or comparable feature, e.g. 3 hand strikes on the table. Do not rate the whole event on an aggression scale.

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Observed consequences / harm

☐ no harm observed
☐ injury to the person
☐ injury to another person
☐ damage to objects
☐ activity interrupted
☐ missing data
☐ other: .....

Actual consequences — description

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Potential risk — separate from actual consequences

What might have happened or was prevented? Do not record it as a consequence that actually occurred.

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6. Timing and readiness

Duration of the described behavior

Give the start and end of the behavior and its duration. Mark estimate / missing data / ongoing at observation end.

Reduction in observed signs of tension

Which signs decreased? How many minutes after the described behavior started was this change observed? Give minutes or clock time. Not observed by observation end / missing data are valid answers.

Time until signs of readiness were observed

How many minutes after the described behavior ended were signs of readiness observed? You may give clock times. Mark estimate / missing data / not observed by observation end. Describe needs and signs of readiness in the After escalation section.

7. After escalation

Sources for this description — After escalation

☐ O — direct observation ☐ P — person's communication (including AAC) ☐ R — another person's report ☐ H — hypothesis

Source details (optional) — After escalation

.....

What did the person need before resuming contact or activity? What was provided, and how was readiness recognized?

Include observations and information communicated by the person. If it is difficult to assess — indicate this.

What happened after the event?

☐ moving to a calmer place
☐ conversation later
☐ sleep
☐ no further demands
☐ rest
☐ return to activity
☐ contact with a close person
☐ other: .....

Was there physical intervention during this event?

☐ Yes ☐ No ☐ Unknown

If so — how many times during this event?

Enter a count or estimate; mark missing data if unknown.

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Did the person react faster or more strongly than usual?

☐ Yes ☐ No ☐ Unknown

Physical intervention: action, context, consequences / safety

Briefly: action, context, consequences, safety and what could change next time. No instructions for physical techniques.

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Environment map

Observation period covered by the map

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Map update date

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Mark sources: O — observation, P — the person's communication (including AAC), R — another person's report, H — hypothesis. The map records current knowledge, preferences and hypotheses. Liking an activity does not demonstrate reinforcement; cooperation does not establish feeling safe; silence does not mean no burden.

Basic information

Date ..... **Time** ..... **Place** .....

Person's identifier ..... **Person completing the record** .....

Other people present

.....

Observation start and end

.....

Environment

☐ Home ☐ School

☐ Care setting ☐ Other: .....

1. Places and preferred spaces

Where is it easiest for the person?

☐ person's room ☐ living room / shared space

☐ kitchen / dining room ☐ bathroom

☐ hallway / transitions ☐ outside

☐ car ☐ other: .....

What do you observe in these places, and what does the person say about them?

Consider quiet, light, control of stimuli, predictability and accompanying people. Separate observation from a hypothesis about what helps.

Which places does the person avoid, or where is it harder?

☐ person's room ☐ living room / shared space

☐ kitchen / dining room ☐ bathroom

☐ hallway / transitions ☐ outside

☐ car ☐ other: .....

What do you observe in these places, and what might make the situation difficult?

Consider noise, smells, crowding, transitions, waiting and lack of influence. Mark hypotheses H.

2. Activities, roles and relationships

What does the person like doing or choose?

Activities, topics, relationships, places or routines. Separate preference from observed signs of recovery.

Which role in an activity is easiest?

☐ leading / controlling ☐ cooperating participant

☐ observer / present without active participation ☐ independently without interaction with others

3. Conditions for optimal functioning

It is easiest for the person when…

- ☐ there is quiet / little auditory input
- ☐ the surroundings are calm
- ☐ they have solitude / time without demands
- ☐ contact is 1:1
- ☐ they influence how the activity unfolds
- ☐ they can negotiate or choose
- ☐ they have eaten / drunk
- ☐ temperature is comfortable
- ☐ other: .....
- ☐ light is dimmed / natural
- ☐ they can control stimuli
- ☐ a close person is present without demands
- ☐ parallel activity is possible without direct communication
- ☐ they know what will happen and how long it will take
- ☐ there are routines / repetition
- ☐ they are rested
- ☐ there is no pain / discomfort

4. Support and regulators

Whom does the person choose to contact? What happens during contact?

Notice contact style: tone, distance, absence of pressure and predictability.

What reduces tension?

- ☐ quiet / reducing stimuli
- ☐ weight / pressure
- ☐ stereotyped movements / rocking / jumping
- ☐ sleep / nap
- ☐ a close person without demands
- ☐ physical contact
- ☐ withdrawing the demand
- ☐ choice / ability to decide
- ☐ other: .....
- ☐ movement — swing / trampoline / walk
- ☐ cold or warmth
- ☐ food / drink
- ☐ toilet
- ☐ solitude / withdrawal
- ☐ distance / personal space
- ☐ access to a preferred activity
- ☐ information about duration

What gives energy / helps restore resources?

Not only what appears calming: record the person's account or observed signs of recovery after effort, separately from hypotheses.

5. What does functioning depend on?

Under which conditions do you observe differences in behavior or readiness?

- ☐ noise
- ☐ change of plan
- ☐ sleep
- ☐ pain / discomfort
- ☐ social contact
- ☐ waiting
- ☐ other: .....
- ☐ number of people
- ☐ time of day
- ☐ food / drink
- ☐ temperature
- ☐ time pressure
- ☐ transitions between activities

How does behavior differ across these conditions?

Describe when it is easier and when signs of tension appear sooner. An observed difference does not by itself establish a cause.

6. Safe spaces and people

Where / with whom can the person return to regulation without additional demands?

Without a repair conversation, questions or new instructions; consider communicated needs and observed signs of readiness. Cooperation is not evidence of feeling safe.

7. Escalation contexts and prevention

When does escalation risk most often increase?

☐ waiting
☐ change of plan
☐ social contact
☐ lack of influence
☐ tiredness / hunger / pain
☐ end of activity
☐ noise / crowding
☐ behavior correction
☐ moving to another place
☐ other: .....

What might reduce escalation risk?

☐ advance notice / preparation
☐ choice of when / how / in what order
☐ clear ending
☐ presence of a close person
☐ other: .....
☐ timer / visual plan
☐ negotiation / discussion
☐ no rush
☐ access to sensory regulation

Do similar situations occur without the described escalation?

☐ Yes ☐ No ☐ Unknown

If so — what is similar or different? What might help?

Consider people, place, predictability, choice and pressure. Give examples and numbers of opportunities if available. Repeated association does not establish causality.

Bibliography accompanying the forms

The references below are preserved from the "Theoretical background" section of Małgorzata Mikołajczyk's guide "How to Use Monitoring Forms". They provide accompanying theoretical background, not evidence of validation of this tool or of the accuracy of every category.

Cooper, J. O., Heron, T. E., & Heward, W. L. (2020). *Applied Behavior Analysis* (3rd ed.). Pearson.

O'Neill, R. E., Albin, R. W., Storey, K., Horner, R. H., & Sprague, J. R. (2015). *Functional Assessment and Program Development for Problem Behavior: A Practical Handbook* (3rd ed.). Cengage Learning.

Garfinkel, S. N., Seth, A. K., Barrett, A. B., Suzuki, K., & Critchley, H. D. (2015). Knowing your own heart: Distinguishing interoceptive accuracy from interoceptive sensibility. *Biological Psychology*, 104, 65–74.

Shah, P., Hall, R., Catmur, C., & Bird, G. (2016). Alexithymia, not autism, is associated with impaired interoception. *Cortex*, 81, 215–220.

Deci, E. L., & Ryan, R. M. (2000). The "what" and "why" of goal pursuits: Human needs and the self-determination of behavior. *Psychological Inquiry*, 11(4), 227–268.

O'Nions, E., Christie, P., Gould, J., Viding, E., & Happé, F. (2014). Development of the "Extreme Demand Avoidance Questionnaire" (EDA-Q): Preliminary observations on a trait measure for pathological demand avoidance. *Journal of Child Psychology and Psychiatry*, 55(7), 758–768.

Substance Abuse and Mental Health Services Administration. (2014). *SAMHSA's Concept of Trauma and Guidance for a Trauma-Informed Approach*. HHS Publication No. SMA 14-4884.